

Orbital walls and fracture routes

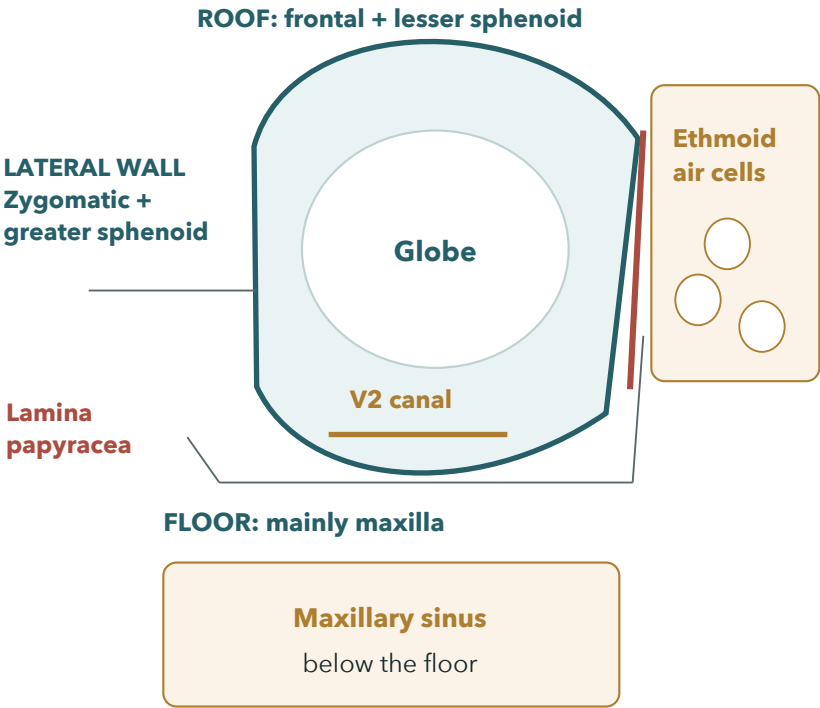
Link a wall or boundary to the sinus, nerve or tissue next to it.

The orbit shares its walls

Right orbit, anterior view

LATERAL

MEDIAL / NOSE



Patient right orbit is shown. Its medial wall is beside the nose; the maxillary sinus lies below its floor.

Location predicts the route

Wall / opening	Adjacent structure
Medial lamina papyracea	Ethmoid air cells; common sinus-to-orbit spread route.
Floor, mainly maxilla	Maxillary sinus; inferior rectus and infraorbital canal above.
Inferior orbital fissure	Infraorbital V2 route into the floor canal.

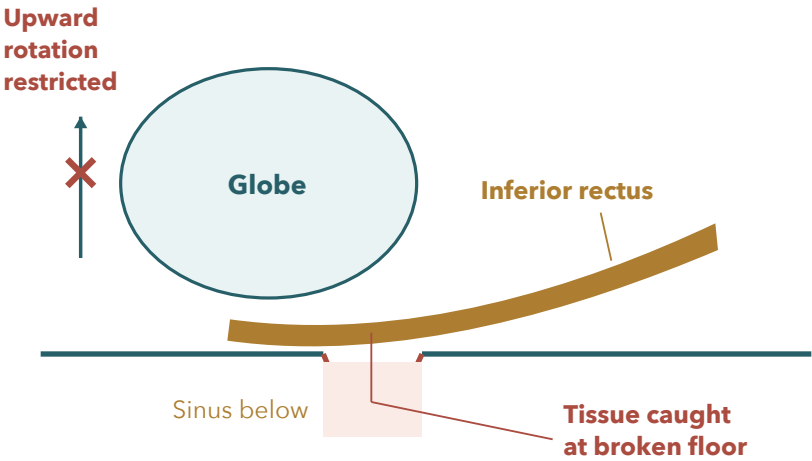
Localise the sensory loss

Infraorbital injury affects lower lid, cheek and upper lip.
Corneal or forehead sensory loss instead suggests V1 involvement.

Tethering and the septal boundary

Why a floor fracture limits elevation

FLOOR FRACTURE: SIDE SECTION



A DIFFERENT BOUNDARY: ORBITAL SEPTUM

PRESEPTAL
Lid tissues

POSTSEPTAL
Orbital fat + muscles

Inferior rectus or nearby connective tissue may be caught at the fracture. The septum inset shows a separate anterior/posterior boundary.

Restriction or denervation?

Finding	Mechanism supported
Passive upward rotation restricted	Mechanical tethering at the floor, rather than isolated elevator weakness.
Active movement weak; passive free	A motor nerve or muscle weakness is more likely.
Proptosis + painful movement	Orbital tissue involvement posterior to the septum.

Two boundaries, two questions

Lamina papyracea separates orbit from ethmoid sinus.
Orbital septum separates superficial eyelid tissues from deeper orbital contents.